SELF-MEDICATION | RESEARCH DASHBOARD

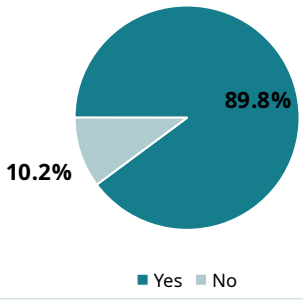
University of Zambia, Ridgeway Campus | Lusale Esther | Planned study window: Aug-Sep 2026; dates not recorded in CSV.

Explore by school | **All schools** | Detailed tables and statistical tests use the full sample.

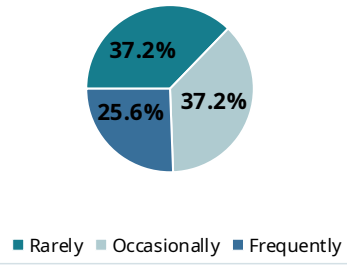
RESPONDENTS	SELF-MEDICATED	PREVALENCE	ANTIBIOTIC USE
383	344	89.8%	49.1%
All valid Yes/No responses	Past 6-12 months	Yes / valid respondents	Among self-medicators only

Prevalence 95% CI: 86.4%-92.5% | Bars show respondent percentages; multi-select totals can exceed 100%.

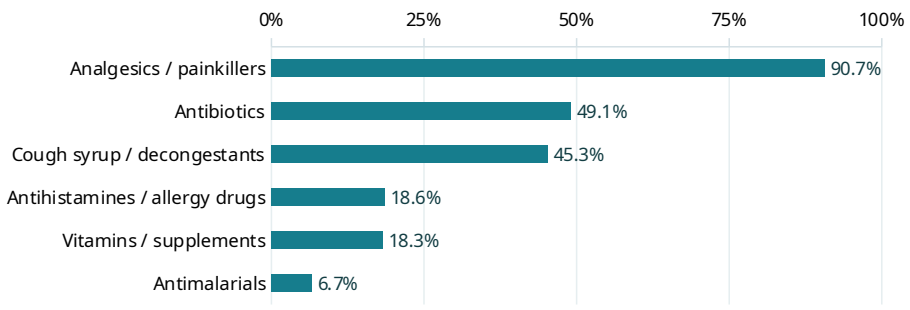
Self-medication prevalence



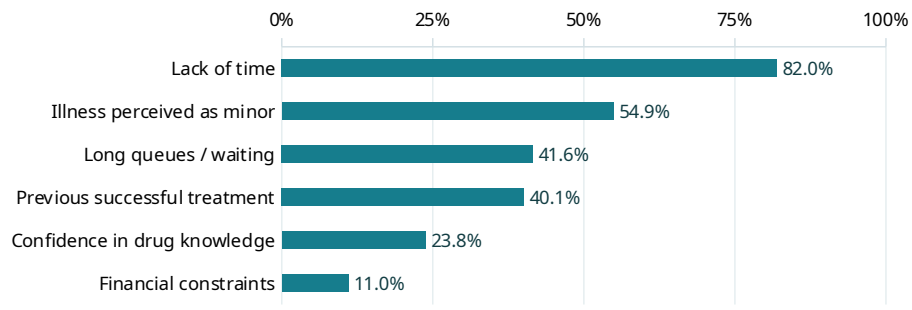
Frequency among self-medicators



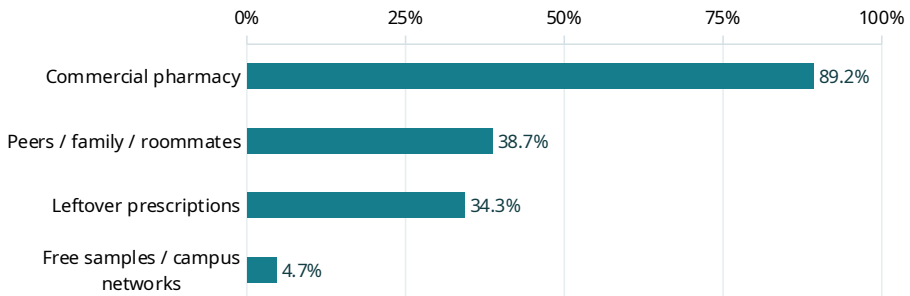
Drug classes | self-medicators



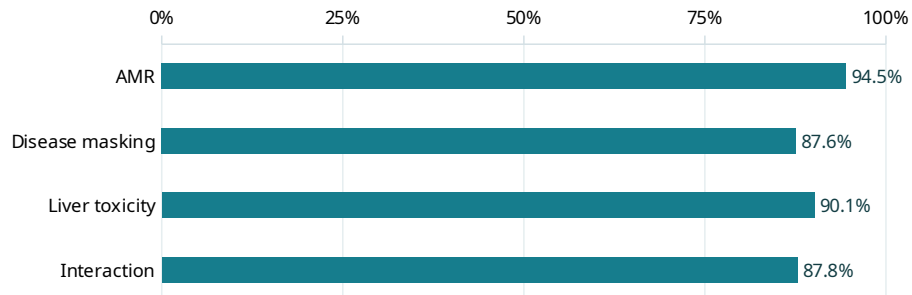
Reasons | self-medicators



Medicine access | self-medicators



Risk awareness | all valid responses



KEY FINDINGS AND INTERPRETATION

Analgesics: 90.7%; antibiotics: 49.1% of selected self-medicators. Time pressure was the leading reported reason.

Full-sample tests: self-rated competence (p=0.0064) and sex (p=0.0425) were associated before multiplicity adjustment; neither met Holm-adjusted p<0.05.

High awareness does not imply safer behaviour. This cross-sectional, voluntary survey cannot establish causes or campus-wide representativeness.

DATA CAUTION: 37/39 No respondents answered Section B. Their drug, symptom, frequency, reason and access answers are excluded from behaviour summaries; original data remain preserved.

HOW TO USE: Change the school dropdown in E5. All six charts and KPI cards update. Detailed tabs remain full-sample; inferential results require rerunning analysis after data edits.

WORKBOOK NAVIGATION

Descriptives | Participant profile & prevalence
Behaviours | Patterns, drugs, reasons & awareness
Associations | Cross-tabs, tests & adjusted model
Methods & Quality | Proposal mapping, definitions & caveats
Cleaned Data | Normalised analysis-ready responses
Raw Data | Original supplied responses

Prepared 7 October 2026 | Sources: LUSALEESTHER-RESEARCHPROPOSALCORRECTIONS.docx and SelfMedicatingPrevelanceCleaned.csv. Planned recruitment target 387; achieved 383 is target attainment, not a response rate.